HALCYON RIDGE HEALTH SYSTEM | ENTERPRISE AI ENABLEMENT ADVISORY | PREPARED BY REGINAL CAMPBELL, PMP

Governance, Risk & Value Realization Framework

Lean oversight proportional to risk, clear decision rights, and value that Finance can verify

Document	HRHS-AI-07	Version 2.1	November 2026 (supersedes v2.0)	Prepared by: Reginal Campbell, PMP
Audience and purpose	Executive sponsors, program and engineering leadership. Define the operating model that governs Documents 05, 06, and 08.
Decision status	Proposed operating model (hypothetical). Takes effect only if the ELT approves D-03. No governance body has been chartered.
Classification	FICTIONAL CASE STUDY. All organizational details, interviews, figures, dates, and findings are simulated scenario data, estimates, or labeled assumptions. No real engagement, approval, or result is claimed. Not a description of any real organization.

Design principle

Oversight scales with risk. Tier 1 uses (administrative, no PHI) are approved in five business days by the Program Office and a privacy analyst. Tier 2 and Tier 3 uses go to the Risk Review Board. Engineering teams keep the technical decisions they are best placed to make. **v2.0 change:** biweekly steering, separate workstream status meetings, and duplicate reporting were removed; gates now fall on the monthly Steering Committee at Days 30, 60, and 90.

Governance bodies

Body	Remit	Chair	Cadence
ELT (existing meeting)	Option, tranches, High risk acceptance, cross-program priorities	CEO	G0, G4, quarterly
AI Steering Committee	Gates G1, G2; escalations; portfolio priority; value review	COO (Executive Sponsor)	Monthly, 60 min
AI Risk Review Board	Tier 2 and 3 approvals; control exceptions; incidents	CPO and CISO; CMIO for Tier 3	Biweekly, 45 min
Dependency review	Cross-team dependencies and impediments	TPM Director	Weekly, 30 min

Decision authorities

Decision	Authority	Not overridable by
Select or change option	ELT (CEO final)	n/a
Tranche release; budget change above $250K	CFO with ELT	Steering Committee
Release of committed ERP/EHR staff; change to ERP/EHR milestones	ERP or EHR executive sponsor with CIO/CTO; ELT if a go-live date moves	AI program
Scope or sequencing within a wave (no tranche change)	Steering Committee on product owner recommendation	Individual teams
Add use case to portfolio	Steering Committee after tiering	Departments
Tier 3 deployment or expansion	CMO on CMIO recommendation	Business sponsors
Halt for privacy or security nonconformance	CPO or CISO	Risk acceptance
Design within reference pattern, tooling, test approach, sprint sequencing, estimates	Engineering managers and teams	Program Office

Decision	Authority	Not overridable by
Accept residual risk	Risk owner up to Medium; ELT for High	Cannot accept bypass of MC-01 to MC-10

Accountability (RACI)

R Responsible, A Accountable, C Consulted, I Informed. PD = AI Program Director (TPM Director); EM = engineering managers; BO = business/product owner.

Activity	CEO	CFO	COO	CIO	CMO	CPO/ CISO	PD	EM	BO
Option decision	A	C	R	C	C	C	C	I	I
Funding and tranches	I	A	C	C	I	I	R	I	C
Capacity commitments	I	C	C	A	I	I	R	R	I
Intake and tiering	I	I	I	C	C	A	R	I	C
Clinical safety (Tier 3)	I	I	I	I	A	C	C	I	R
Use case delivery	I	I	A	C	I	C	R	R	R
Technical design	I	I	I	A	I	C	C	R	C
Benefits measurement	I	A	C	I	I	I	R	I	R
Production monitoring	I	I	I	A	I	C	C	R	R

Escalation thresholds

Trigger	Escalate to	Timing
Validated capacity found unavailable in the delivery window, or contractor onboarding misses date	Executive Sponsor and CIO/CTO notified; Steering Committee decides Document 08 options	Notify within 1 business day; decide at next Steering
Gate date at risk by more than 2 weeks	Steering Committee	Next meeting or ad hoc
Workstream variance above 10% or $100K	Steering Committee	Next meeting
Variance above $250K, or wave scope change	CFO and ELT	Within 10 business days
Suspected PHI privacy incident	CPO under breach procedure	Within 24 hours
Security incident affecting AI services	CISO under incident plan	Immediate
Tier 3 safety event or near miss	CMIO; use paused	Immediate
Output quality below threshold two periods running	Product owner and Risk Review Board	Within 5 business days

Change control and baselines

Baselines (scope, schedule, budget, capacity) are set at G1 and changed only at gates. Minor changes within workstream tolerance: Program Director with engineering manager. Moderate (affects a gate or more than 10% of workstream budget): Steering Committee. Major (option, tranche, wave scope, or ERP/EHR milestones): ELT.

Regulatory obligations versus governance practice

Instrument	Status	How it applies at HRHS
HIPAA Privacy and Security Rules	Legal obligation	Applies wherever PHI is used (P-01, P-02, Wave 2; supply chain only if patient-linked data such as implant records is included).

Instrument	Status	How it applies at HRHS
Proposed HIPAA Security Rule update (Jan 2025 proposal)	Not final	Planning input only; HHS has projected final action for July 2027.
Section 1557, 45 CFR 92.210	Legal obligation where applicable	Applies to patient care decision support tools used by covered entities. Documentation and revenue cycle tools are likely outside its scope; counsel confirms per tool.
FDA device rules for clinical software	Legal where a tool is a device	Determined per clinical tool by regulatory counsel; not relevant to supply chain or revenue cycle use cases.
NIST AI RMF 1.0 and Generative AI Profile (AI 600-1)	Voluntary	Adopted as HRHS practice for the Govern, Map, Measure, Manage structure.
State AI and privacy laws	Varies	Reviewed by counsel for each Tier 2 and 3 use.

Model monitoring

Pre-deployment evaluation on representative HRHS data; subgroup review for Tier 3 and patient-impacting Tier 2. In production: sampled human review at a rate set by tier, acceptance and override rates, vendor model-change notices logged, AI-specific incident categories in service management, and re-certification annually or on material change.

Value validation

Rule	Practice
Business ownership	Each benefit has a business owner; Finance validates baselines before a pilot and benefits before reporting.
Benefit types kept separate	Cost savings (UC-02), revenue recovery (UC-04), cost avoidance, and unmonetized risk reduction are never summed into one figure.
Gross versus net	Report gross and net (after rework, licensing, support) separately.
Cash timing	Revenue recovery reported when collected, not when a denial is prevented.
Attribution	Pre/post with logged sourcing events and, where feasible, comparison categories or sites; shared benefits reported as contributing.

Continue, revise, expand, or stop

Action	Criteria
Continue	Acceptance criteria met; controls conform; trend toward benefit target.
Revise	Partially met with a bounded fix; one revision cycle before stop.
Slow	Capacity or dependency shortfall: sequence scope (Document 08) rather than compress quality gates.
Expand	Continue criteria met for two periods; Finance-validated benefits; capacity committed; re-tiered.
Stop	Criteria unmet after revision; unresolved nonconformance; unresolved safety event; or benefits not plausible at full cost. Execute exit plan (MC-10).

Decision and risk acceptance logs

The Program Office keeps a decision log (ID, date, decision, options, rationale, approver) starting with D-01 to D-05 and D-08 series, and a risk acceptance log (risk, residual rating, accepting authority, conditions, expiry at 12 months or material change). Neither log may record a bypass of mandatory controls.

Executive scorecard

ID	Measure and definition	Owner	Baseline method	Target approach	Cadence	Type
L1	Inventory coverage: % of identified AI uses registered and tiered	AI Program Director	Day 1 to 30 survey and procurement scan	v1 by Day 30; 100% by Day 60	Monthly	Leading
L2	Capacity delivery: validated vs actual hours by skill and source	CIO/CTO	D-05 validated allocations (Day 20)	Within 10%	Biweekly	Leading
L3	Data readiness: % of target item records complete and unique	Data Governance Lead	Day 31 profiling (est. 82%)	Set at G2	Biweekly	Leading
L4	Output acceptance: % accepted by reviewer without material change	Product owner	First 2 pilot weeks	Set in acceptance criteria	Weekly	Leading
L5	Control conformance: % of deployed uses with MC-01 to MC-10 evidenced	CISO	G1 audit of P-01, P-02	100%; exceptions time-limited	Monthly	Leading
L6	Intake cycle time: median days intake to tier decision	CPO	First 20 intakes	5 days Tier 1; 10 days Tier 2/3	Monthly	Leading
O1	On-contract spend rate	VP Supply Chain	Validated 12-month rate (est. 71%)	Movement toward 85%; attribution agreed with Finance	Quarterly	Lagging
O2	Preventable denial write-offs ($), net recovery collected	VP Revenue Cycle	Root-cause sample by Day 60	Set in P-02 or Wave 2 case	Quarterly	Lagging
O3	Documentation time per encounter (P-01)	CMIO	EHR audit logs vs matched clinicians	Only if P-01 continues	Quarterly	Lagging
O4	AI incidents by severity	CISO and CMIO	Count from Day 1	Zero unresolved high-severity	Monthly	Lagging
O5	Program spend vs authorized baseline	CFO delegate	Tranche 1 plan	Within 10%	Monthly	Lagging
O6	Finance-validated net benefits vs case range	CFO	Restated case at G4	Within stated range by month 12	Quarterly	Lagging